MRL - Milestone Review Log

MRL - v0.2

Site Name / Code	Protocol Number	Principal Investigator	Date Opened
Reviewer and Log Owner	Role and Title	Study Phase and Site Status	Current Review Cycle
Review Period Covered	Linked POT Version / Oversight Plan Reference	Sponsor and CRO Contact	Key Source Systems / Logs Used

Record Type

■ Planned Milestone Review ■ Unscheduled Trigger Review Event

A Purpose and Use of This Log

The Milestone Review Log is a site-level QMS record used to demonstrate that PI oversight is performed at defined control points and when relevant trigger events occur. It links the PI Oversight Template, delegation structure, device accountability process, safety surveillance, critical data review, CAPA management, and inspection-readiness evidence into one traceable review record.

The log supports a tiered, risk-adjusted model of oversight. The PI does not need to re-perform every delegated task or review every routine source document at every cycle. Delegated staff may prepare simplified summaries, trackers, exception reports, dashboards, or review packets when these accurately represent the underlying records and clearly identify open issues. However, when the summary identifies an unusual event, unresolved discrepancy, clinically meaningful deviation, safety concern, device accountability issue, endpoint risk, or data inconsistency, the PI should review the relevant raw data or source documents directly when needed to make an informed decision.

This document should therefore be used both as a scheduled governance tool and as an escalation record. If the review is planned, complete the Planned Milestone Review Log and reference the applicable milestone ID. If the review is triggered by an out-of-ordinary event, use the Unscheduled Trigger Review Record and document the trigger, immediate containment action, PI risk assessment, reporting decision, and closure status. Examples of trigger events include involvement of a non-credentialed or non-released surgeon, missing implant lot number or UDI, delayed SAE awareness or reporting, missed critical imaging or endpoint window, and revision, explant, reoperation, readmission, or serious complication.

B Planned Milestone Review Matrix

ID	Milestone	Timing / Periodicity	Minimum Items to Review	Expected Evidence / Output
M0	Site feasibility / capability confirmation	Before site activation or final site acceptance.	PI experience, surgical volume, patient/referral base, imaging capability, OR/device workflow, staff capacity, follow-up pathway, external data access.	Capability decision, open gaps, mitigation plan, PI confirmation.
M1	Site initiation and team qualification	At SIV or before first trial activity.	Delegation log, role-specific training, device release, OR staff readiness, source standards, device storage/accountability setup, escalation pathway.	Initiation readiness sign-off, training gaps, activation restrictions if needed.
M2	First consent / first screening review	After first consent or first screening, before first implantation when possible.	Consent version, process documentation, eligibility source evidence, imaging basis for screening, screening failure logic, reconsent risk.	PI screening/consent review outcome and corrective actions.
M3	First enrolled participant review	After first enrollment and before or shortly after first procedure.	Eligibility confirmation, baseline imaging, baseline PROM/ROM, source completeness, participant safety considerations, schedule for first endpoint windows.	PI enrollment confirmation, open data gaps, follow-up controls.
M4	First surgery / first device-use review	Within the defined post-procedural window after first device use.	Implant configuration, UDI/lots, operative note, deviations, component substitutions, immediate imaging, device deficiencies, early complications.	PI procedure review, device accountability check, event/deviation assessment.
M5	Early case-series / learning curve review	After first 3-5 cases per surgeon/device, or as defined in CIP/site plan.	SLCT data, operative time trend, case complexity, proctoring/support, technical deviations, early complications, imaging adequacy, surgeon competence signals.	Continue, intensified review, retraining, proctoring, or case restriction decision.
M6	Periodic C2 PI oversight review	Monthly by default, or risk-adjusted according to enrollment and site activity.	Consent deviations, device accountability, staff training/delegation currency, open CAPAs, weak signals, critical queries, follow-up at risk, external data pending.	Periodic PI review record, actions, escalations, C2 issue status updates.
M7	Safety / complication signal review	Monthly, quarterly, or immediately after signal threshold is met.	AE/SAE/SADE/USADE, device deficiencies, complication clusters, readmission, reoperation, explant, revision, delayed reporting, causality patterns.	Signal assessment, PI clinical interpretation, reporting/action decision.
M8	Amendment implementation readiness review	Before implementing any approved substantial change or relevant new version.	New CIP/ICF versions, changed eligibility, procedure, endpoint, imaging, device workflow, safety reporting, training and delegation updates.	PI readiness confirmation, implementation date, restrictions, training evidence.
M9	Follow-up and endpoint-window readiness review	Before primary endpoint windows and periodically during active follow-up.	Visit windows, missing imaging, PROM/ROM completion, interval events, retention risk, lost-to-follow-up risk, endpoint-critical assessments.	Endpoint readiness status, recovery actions, PI assessment of data impact.
M10	Critical data / external data reconciliation review	Before data locks, endpoint analysis, monitoring visits, or as pending data accumulates.	Outside imaging, external hospital records, readmission records, reoperation reports, ePRO/PROM outputs, critical queries, source-to-eCRF consistency.	Reconciliation status, unresolved data list, PI interpretation decisions.
M11	Pre-closeout device accountability review	Before site closeout or final device reconciliation.	Device receipt/use/return/destruction, UDI/lots, unused stock, discrepancies, device deficiencies, participant-device traceability, accountability log completeness.	Device reconciliation outcome, discrepancy resolution, PI sign-off.
M12	Post-investigation quality debrief / CAPA handover	After final participant visit, closeout, or major study completion milestone.	Deviation patterns, safety signals, missed assessments, follow-up loss, device issues, CAPA effectiveness, lessons learned for future investigations.	Quality debrief summary, CAPA handover, improvement actions.

The timing above may be adapted if the CIP, sponsor oversight plan, ethics requirements, or site risk profile requires a more intensive cadence.

C Planned Milestone Review Log

Date	Milestone ID	Subject(s) / Period / Case(s)	Evidence Reviewed	Outcome / Decision / Action Required	PI Initials / Date

Use this page when the review type selected on page 1 is Planned Milestone Review. If the record type is Unscheduled Trigger Review Event, this planned log page may remain blank.

D Unscheduled Trigger Review Event Record

Trigger Date / Time		Date PI Notified	
Detected By		Role / Source of Detection	
Trigger Category	■ Safety ■ Device deficiency ■ Protocol deviation ■ Eligibility ■ Device accountability ■ Endpoint risk ■ Surgeon release ■ Other		
Subject, Case, Surgeon, Device			
Trigger Description			
Immediate Safety or Containment Action			
PI Risk Assessment			
Criticality Level	■ C1 ■ C2 ■ C3	Device and Procedure Relationship	■ Related ■ Possibly related ■ Not related ■ Undetermined
Reporting Required?	■ Sponsor ■ CRO ■ Ethics ■ CA ■ Not required ■ Pending	Deadline or CIP Timeline	
Decision / Escalation / CAPA			
Status	■ Open ■ Under review ■ Escalated ■ Closed	Closure Date and PI Initials	

Use this page when the review type selected on page 1 is Unscheduled Trigger Review Event. If the record type is Planned Milestone Review, this page may remain blank.

E PI Review Cycle Certification

I certify that the milestone reviews and triggered reviews documented in this log were performed according to the site PI Oversight Template and the applicable Clinical Investigation Plan. I confirm that relevant delegated summaries, source records, raw data, exception reports, and supporting evidence were reviewed at a level appropriate to the risk and criticality of the issue, and that identified actions, escalations, CAPAs, or closure decisions have been documented to the best of my knowledge.

Principal Investigator Signature	Date
Reviewer and Log Owner Signature	Date
Outstanding Actions at Certification	Next Review Due or Follow-up Required